



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Procedure

Electroconvulsive Therapy Coordinators Role

Scope

Site	Service/Department/Unit	Disciplines
Fremantle Hospital	Mental Health Service	Nursing

1. Introduction

This document outlines the role and responsibilities of nursing staff in the role of Electro Convulsive Therapy (ECT) Coordinator to ensure ECT is provided in accordance with the [Mental Health Act \(MHA\) 2014](#), [the Chief Psychiatrists Standards for the Administration of ECT in WA](#) and the [Chief Psychiatrists Guidelines for the Administration of ECT in WA](#).

2. Terminology

Consent to (medical) treatment	In the context of health care, consent is a patient's agreement that a health practitioner can proceed to perform a specific proposed treatment.
Electroconvulsive Therapy (ECT)	The application of electric current to specific areas of the head to produce a generalised seizure which is modified by general anaesthetic and the administration of a muscle relaxing agent.
Electroencephalogram (EEG)	Technique for recording the electrical activity from different parts of the brain and converting into a tracing.
South Health Information Management System (SHIMS)	A Structured Query Language (SQL) database driven web application used for health information management. The Fremantle Hospital ECT Register is maintained in SHIMS.

3. ECT Coordinator

The ECT Coordinator must have completed a recognised ECT training program.

The ECT Coordinator provides nursing leadership and role modelling to advance best practice in ECT by:

- providing direct patient care and supervision for the provision of ECT treatment
- promoting, developing and implementing standards and/or policies related to ECT in collaboration with ECT Clinical Nurse Specialist
- orientating nursing staff to the ECT service
- assessing, planning, implementing and evaluating quality improvement, clinical audit and education programs relevant to ECT
- providing specialist knowledge of ECT equipment and treatment techniques.
- supporting and evidencing the application of the Chief Psychiatrists Standards for the administration of ECT in WA to maintain service approval to provide ECT.
- applying advanced problem-solving strategies to resolve technical and procedural issues.
- coordinating a multidisciplinary team (MDT)
- escalating clinical concerns and patients of concern to ECT Clinical Nurse Specialist and ECT Lead Psychiatrist

4. ECT Coordinator role

4.1. ECT Bookings

- Ensure ECT bookings are made via eReferral, with the ECT Coordinator working in collaboration with the ECT Clinical Nurse Specialist as the Gatekeeper
- Following receipt of referral, the ECT Coordinator places the patient onto the Booking Assistant Scheduling Engine (BASE).
- The ECT Coordinator will ensure all documentation is complete including:
 - ECT Consent form
 - Anaesthetic consent form
 - ECT/Anaesthetic Integrated form
 - Anaesthetic record
 - WA Hospital Medication chart

- Pre ECT Checklist
- For involuntary patients, Mental Health Tribunal approval and Mental Health Act 2014 forms must be available
- Ensure education has been provided or offered to the patient and/or family.
- Advise the FH Theatre Coordinator if the ECT list is cancelled.
- Inform the rostered Anaesthetist of late bookings (<24 hours' notice) and/or if anaesthetic review is required.

4.2. Equipment Preparation

Nursing staff working in the role of ECT Coordinator should have a working knowledge of the equipment in the ECT procedure room and recovery room, and have the appropriate knowledge, skills and experience to use of the following equipment:

- Thymatron IV™ Machine
 - Resuscitation equipment
 - Cardiac and vital signs monitors
- Check all resuscitation equipment e.g.: laryngoscopes, all suction and oxygen are available and working.
 - Turn on the Thymatron IV™ Machine, test impedance, and check paper level and printer speed is set at 10cm: 10 seconds as per [Appendix 1](#)
 - Provide revision of Thymatron IV™ Machine use and safety to rostered Registrar if required.
 - Turn on cardiac and vital sign monitors (including recovery room).
 - Ensure sufficient anaesthetic masks, bite blocks, Cobb's connectors and airway filters are available.
 - Retrieve previous treatment information from the ECT treatment register.
 - Ensure the Theatre Management System (TMS) is commenced for the patient.
 - Ensure intravenous cannulation equipment is available.
 - Collect medication keys from Ward W4.2 Schedule 4 locked cupboard.
 - Make medications available to the Anaesthetist.

4.3. Patient Preparation

- Day procedure patients will be admitted directly to the ECT Suite.

- Note any medication or devices that may affect high / low heart rate or seizure threshold and alert the Psychiatrist
- Obtain verbal patient consent if any staff and/or students wish to observe ECT.

4.4. ECT Treatment

- The ECT list commences at 08:00 when the Consultant Psychiatrist, Psychiatric Registrar, Anaesthetic Consultant, Anaesthetic Technician and Recovery Nurse are all present.
- Clinical handover with the Anaesthetist and Psychiatrist will include information relating to the previous treatments and/or any current issues that may impact on treatment.
- Receive clinical handover from escort nurse using the iSoBAR format.
 - **Identify-** Two nurses are to confirm the identity of the patient using a minimum of three approved patient identifiers, of which the following are examples.
 - Patient name (family and given)
 - Two standard patient identification bands
 - Date of birth
 - Unit Medical Record Number (UMRN)
 - Photographic ID
 - Address
 - **Situation-** Check consent is valid. Confirm patients' understanding of ECT and verbal consent prior to each treatment to confirm:
 - time patient fasted
 - medications taken
 - allergy status
 - dentures/crowns/loose teeth
 - previous mastectomies and pacemakers
 - **Observation-** Check pulse, blood pressure, respiratory rate, BGL, oxygen saturation, temperature are documented prior to treatment.
 - **Background** -Advise Anaesthetist of baseline observations and medications given.
 - **Agreed plan-** If fasting time is inadequate or medications given late a decision to cancel treatment may be made by the Anaesthetist. The ECT Coordinators will follow up with relevant ward managers.
 - **Read back-** essential information from handover

The ECT Coordinator will confirm the ECT Consent:

- has been reviewed if, after consent was obtained the patient's circumstances have changed
- has been renewed after either 12 treatments, change in placement or timeframe whichever comes first.
- for a patient receiving treatment under the MHA 2014 is the documented Mental Health Tribunal decision of approval available and expiry date is written on prescription

- Confirm ECT percentage dose required with Registrar and psychiatrist and liaise with psychiatrist to set the dosage on the Thymatron IV™ Machine.
- Perform hand hygiene
- Ensure the patient's forehead and the area behind both ears is wiped with skin prep and gauze.
- Place electrodes on patient and connect patient to Thymatron IV™ Machine. Refer to [Appendix 2](#)

Trouble shooting guide if impedance is high (3000 and over):

- Apply conductivity gel to the hairline and electrodes
- If still problematic re-clean the forehead and reapply new electrodes
- If contact with the thymapad stimulus electrodes is not satisfactory then the Registrar may utilise the paddles and the Consultant Psychiatrist will activate the Thymatron IV™ Machine whilst the Registrar holds paddles in place.

- Collect Thymatron IV™ baseline by pressing impedance and obtaining a printout. While collecting the baseline request the patient to remain still with eyes closed to ensure quality reading.
- Check the quality of the reading is satisfactory.
- The ECT Coordinator leads Team Time Out immediately prior to treatment with the Psychiatric Registrar, Consultant Psychiatrist, Consultant Anaesthetist and anaesthetic technician confirming the following:
 - Patient identification
 - Anaesthetic monitoring is confirmed

- Electrode position
- Treatment dose
- Baseline has been completed
- Impedance
- Pulse width
- Team Time Out is documented by the ECT Coordinator
- The Anaesthetist is then advised pre-oxygenation may commence.
- The two-minute countdown begins once Suxamethonium has been administered. The time may vary according to individual patient requirements.
- Observe for fasciculation and inform the ECT team when the fasciculation has ceased, or two minutes is up whichever comes first.
- Ensure bite block is placed and in correct position before anaesthetist confirms patient is anaesthetised and ready for treatment.
- Time the seizure using the Thymatron IV™ Machine timer and confirm verbally with ECT team
- On completion remove Thymatron IV™ leads from the patient.
- Perform hand hygiene.

4.5. Patient flow

- Ensure smooth flow of the list by requesting patients are transferred to the ECT Suite in a timely manner.
- Decontaminate leads, attach new electrodes to connections prior to the next patient entering treatment room.

Thymatron™

- The Thymatron IV™ machine must be reset for each patient
- The next patient must not be allowed into the ECT treatment room until all documentation for the previous patient is completed and filed.

- Ensure psychiatrist and registrar have documents treatment data in SHIMS ECT database

4.6. Post ECT procedure

- The ECT Coordinator will coordinate post procedure care for each patient including:

- anaesthetic review for any patient requiring further management.
- liaising with inpatient treating team and ward staff
- ensuring cognitive assessment is completed 30 minutes post ECT.
- liaising with the Community Treatment Teams for ECT outpatients to ensure continuity of care

4.7. Completion of ECT List

- Check and document the Schedule 4 Restricted Drugs with another nurse.
- Turn off all equipment in the ECT suite and recovery room.
- Restock consumable equipment.
- Re-order non imprest stock as necessary
- Check expiry dates and reorder pharmacy supplies
- Print the clinical handover report from SHIMS for scanning into the Digital Medical Record.
- Liaise with treating teams regarding new consent or treatment/prescription if due for renewal.
- Ensure bookings are complete and patients are contacted in preparation for next list
- Order breakfast for outpatients/day procedure patients

4.8. High risk patients – FH Operating Theatre

- Patients assessed as high risk by the Anaesthetist will have ECT performed in the FH B Block Operating Theatre Complex.
- Arrange for the Thymatron IV™ Machine to be delivered to the Operating Theatres B Block Level 5.
- All patients going to the operating theatre must wear two ID bands (eg. wrist and ankle) as appropriate.

5. Compliance Monitoring

ECT Clinical Nurse Specialist to complete monthly statistics on completed ECT courses including emergency ECT (Form 13 - Statistics about ECT) and provide to the Head of Service.

Clinical incidents notified in Datix will be regularly monitored by the ECT Clinical Nurse Specialist and Nurse Unit Manager.

ECT Coordinator in collaboration with the ECT Clinical Nurse Specialist will perform clinical audits on all completed courses.

6. Related Policy Documents

- WA Health:
 - [Clinical Incident Management Policy](#)
 - [Consent to treatment](#)
- SMHS
 - [Hand hygiene & Skin Integrity \(SMHS: 245\)](#)
- FSFHG:
 - Cognitive screening of the older person
 - Observation and response chart
 - Clinical Handover
 - Patient identification
 - Clinical documentation
 - Electroconvulsive Therapy
 - Electroconvulsive Therapy Registrar Role
 - Electroconvulsive Therapy Recovery and discharge
 - Environmental cleaning – Fremantle Hospital
 - Pre and post procedure patient management

7. Related policy documents

- [Royal Australian New Zealand College of Psychiatrists \(RANZCP\) Professional Practice Guidelines for the Administration of Electroconvulsive Therapy.](#)

8. Related Standards

- NSQHS Standard:
 - Clinical Governance
 - Partnering with Consumers
 - Preventing and Controlling Infection
 - Medication Safety
 - Comprehensive Care
 - Communicating for safety

- Chief Psychiatrists:
 - [Standards for Clinical Care](#)
 - [Practice Standards for the Administration of Electroconvulsive Therapy 2025.](#)

9. Related documents

- [MHA 2014](#)

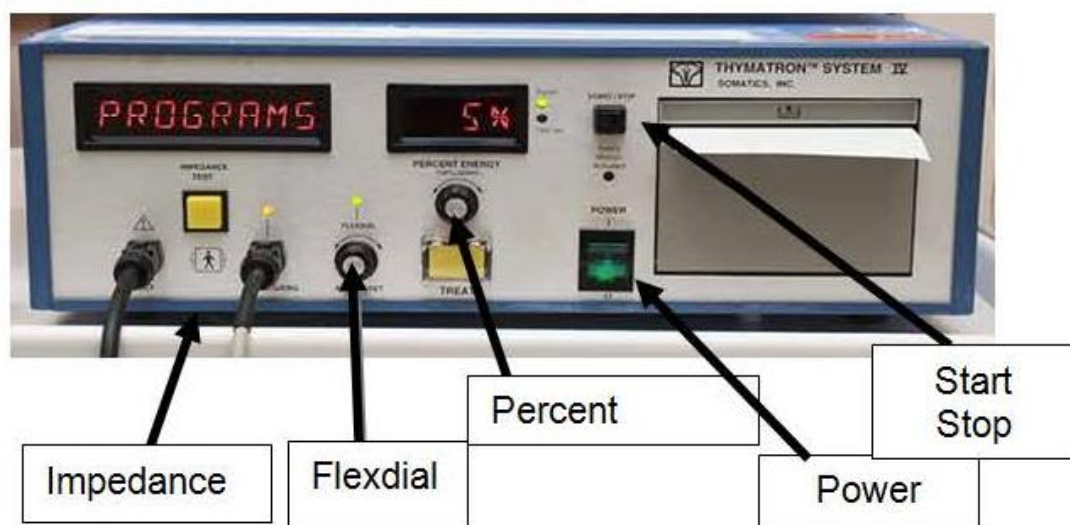
10. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	05/2016	ECT Coordinator	MH Clinical Governance Committee	Nursing Practice Committee	05/2019
2	09/2019	ECT Coordinator	ECT Working Group NMPGC	FSFHG Policy Committee	09/2022
3	10/2022	ECT Coordinator	ECT Meeting NMPGC	Service 5 Safety Quality and Risk Committee	10/2025
4	03/2026	ECT Coordinator	ECT Committee	Service 5 Safety Quality and Risk Committee	03/2029

11. Appendices

11.1. Appendix 1: Thymatron IV™ Machine User Instructions

Figure 1: Thymatron IV™ Machine



- Press the green power button to turn machine on
- Press impedance button to read 220-230
- Press the flexidial button and turn to print out.
- Press the flexidial again and turn flexidial to 10
- Press flexidial to enter printout speed
- Press impedance to escape
- Press stop / start button to check printout speed is slow.
- Press stop / start button again to finish test
- Press and turn the percentage button simultaneously to set the appropriate program, then the impedance button to escape
- Check the paper supply in the Thymatron IV™ Machine and replace if red line is visible
- Open the paper compartment and remove the remaining paper. Insert new paper with the red line on the bottom of the left-hand side and the black square is on the top right-hand side.

11.2. Appendix 2: Thymatron IV™ EEG and ECG lead placement

Figure 2. Bifrontal lead placement

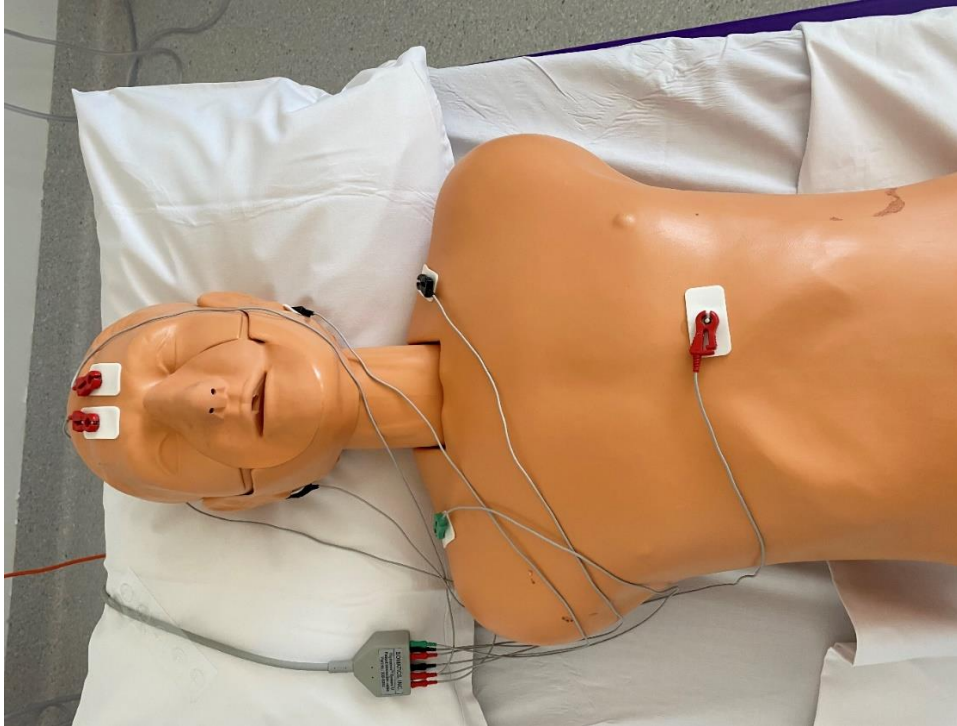


Figure 3. Bitemporal /Right unilateral lead (RUL) placement

